

REDWOOD UNIVERSITY HOSPITAL

300 Sand Hill Rd, Menlo Park, CA 94025 | (650)555-6604 | www.redwooduh.org

DISCHARGE SUMMARY

Patient Name: Ekaterina Volkova	DOB: April 29, 1956 (Age 69)
MRN: MRN-9078533	Gender: Female
Admission Date: February 19, 2025	Discharge Date: March 08, 2025 (17 days)
Attending Physician: Dr. Takashi Mori, MD — Oncology	Insurance: BCBS PPO #BCB-D4-44444
ICU Admission: YES — MICU/SICU	ICU Days: 11 of 17 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Hypertensive Emergency with End-organ Damage	I16.1	Acute / Critical
Metastatic Non-Small Cell Lung Cancer	C78.9	Chronic / Complicating
Severe COPD — FEV1 28% Predicted	J44.1	Chronic / Complicating

HOSPITAL COURSE

Ekaterina, a 69-year-old female with a history of Metastatic Non-Small Cell Lung Cancer and Severe COPD — FEV1 28% Predicted, was admitted following acute-on-chronic renal failure requiring urgent haemodialysis initiation. Pre-existing CKD stage 4 had acutely decompensated in the setting of NSAID use and IV contrast exposure. The patient underwent emergent tunnelled haemodialysis catheter placement under fluoroscopy. Haemodialysis was initiated three times weekly. Hyperkalaemia (K+ 6.8 mEq/L) was managed with IV calcium gluconate, insulin-dextrose, and sodium bicarbonate prior to first session. Renal recovery was partial; the patient was discharged on chronic outpatient haemodialysis three times weekly with nephrology follow-up.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
94/60 mmHg	108 bpm	96% (on 3L O2)	99.8°F	20/min	133 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Rivaroxaban	20 mg	Once daily with evening meal	Indefinite — PE management
Metronidazole	500 mg	Every 8 hours	7-day course
Propofol	Per protocol	Continuous IV sedation	Discontinued prior to discharge
Levetiracetam	1500 mg	Twice daily	Ongoing — neurology follow-up
G-CSF (Filgrastim)	480 mcg SC	Once daily	Until ANC > 1.0 x10 ³ /μL
Norepinephrine	Per protocol	Continuous IV infusion	Weaned and discontinued in hospital

FOLLOW-UP PLAN

URGENT: Oncology: Dr. Takashi Mori, MD — within 72 hours of discharge (March 11, 2025).

Primary Care: Within 1 week (March 15, 2025). Full labs including BMP, CBC, LFTs.

Home health nursing: Daily vitals and wound check for first 7 days post-discharge.

Inpatient rehabilitation / LTACH: Transfer arranged — starts March 18, 2025.

■ **HIGH-RISK DISCHARGE** — Patient requires close monitoring. Re-admission risk: **HIGH**.

Attending Physician Signature: _____

Dr. Takashi Mori, MD — Oncology | License #: CA-MD-54678 | Date: March 08, 2025

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